

Population-level analysis of Coagustat bleeding risk (no individual case)

Abstract. We analysed a registry of 40,000 patients receiving Coagustat to estimate major bleeding rates. This is a cohort analysis and does not describe any identifiable individual patient.

Methods. Retrospective cohort. Major bleeding defined per ISTH criteria.

Results. The annualised major bleeding rate was 3.1 per 100 patient-years, higher in patients over 75 and with renal impairment.

Conclusion. Bleeding risk should be weighed against thrombotic benefit. No case narratives are included.

References. 1. Trial group. Lancet. 2022.